

EQUINOX MEDICAL CENTER

250 Qubit Rd, San Jose, CA 95110 | (408)555-6609 | www.equinoxmc.org

DISCHARGE SUMMARY

Patient Name: Josefina Reyes-Mendoza	DOB: August 19, 1976 (Age 49)
MRN: MRN-7416432	Gender: Male
Admission Date: February 08, 2025	Discharge Date: February 17, 2025 (9 days)
Attending Physician: Dr. Preethi Nair, MD — Infectious Disease	Insurance: Oscar EPO #OSC-I9-99999
ICU Admission: YES — MICU/SICU	ICU Days: 6 of 9 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Status Epilepticus — Refractory	G41.9	Acute / Critical
Coagulopathy — INR 4.8	D68.9	Chronic / Complicating
Cirrhosis — Child-Pugh Class C	K74.60	Chronic / Complicating

HOSPITAL COURSE

Josefina, a 49-year-old male with a history of Coagulopathy — INR 4.8 and Cirrhosis — Child-Pugh Class C, was admitted following acute-on-chronic renal failure requiring urgent haemodialysis initiation. Pre-existing CKD stage 4 had acutely decompensated in the setting of NSAID use and IV contrast exposure. The patient underwent emergent tunnelled haemodialysis catheter placement under fluoroscopy. Haemodialysis was initiated three times weekly. Hyperkalaemia (K+ 6.8 mEq/L) was managed with IV calcium gluconate, insulin-dextrose, and sodium bicarbonate prior to first session. Renal recovery was partial; the patient was discharged on chronic outpatient haemodialysis three times weekly with nephrology follow-up.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
95/69 mmHg	121 bpm	94% (on 4L O2)	100.5°F	21/min	81 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital
Metronidazole	500 mg	Every 8 hours	7-day course
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Rivaroxaban	20 mg	Once daily with evening meal	Indefinite — PE management
Cefepime	2g IV	Every 8 hours	Completed in hospital — 7 days

FOLLOW-UP PLAN

URGENT: Infectious Disease: Dr. Preethi Nair, MD — within 72 hours of discharge (February 20, 2025).

Primary Care: Within 1 week (February 24, 2025). Full labs including BMP, CBC, LFTs.

Home health nursing: Daily vitals and wound check for first 7 days post-discharge.

Inpatient rehabilitation / LTACH: Transfer arranged — starts February 26, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Preethi Nair, MD — Infectious Disease | License #: CA-MD-09123 | Date: February 17, 2025

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